

OREGON STATE HOSPITAL

POLICY

SECTION 7: Patient Rights **POLICY: 7.008**

SUBJECT: Patient Abuse or Mistreatment
Allegation

POINT PERSON: Director of Legal Affairs

APPROVED: Dolores Matteucci
Dolores Matteucci
Superintendent

DATE: FEBRUARY 16, 2024

SELECT ONE: ☐ New policy ☒ Minor/technical revision of existing policy
☐ Reaffirmation of existing policy ☐ Major revision of existing policy

PLEASE NOTE THAT ALL CHANGES WILL BE MADE AS TRACK CHANGES. TO CLEAR AND ACCEPT YOUR TRACK CHANGES, CONTACT OSH.POLICIESANDPROCEDURES@DHSOHA.STATE.OR.US.

I. PURPOSE AND APPLICABILITY

- A. This policy sets forth the mandatory patient abuse and mistreatment reporting obligations and process for all Oregon State Hospital (OSH) staff, consistent with federal and state requirements.
- B. This policy applies to all staff, including employees, volunteers, trainees, interns, contractors, vendors, and other state employees assigned to work at OSH.

II. POLICY

- A. Every patient at OSH deserves safe, respectful, and dignified treatment. To that end, all staff must conduct themselves in such a manner that protects patients from abuse.
- B. Consistent with ORS 430.735, conduct that meets the definitions of abuse or mistreatment is prohibited at OSH and includes, but not limited to:
 - 1. Abandonment, including desertion or willful forsaking of a patient or the withdrawal or neglect of duties and obligations owed a patient by staff;
 - 2. Physical harm to a patient caused by other than accidental means, self-defense, or that appears to be at variance with the explanation given of the injury by staff;
 - 3. Willful infliction of physical pain or injury;
 - 4. Neglect;

5. Verbal abuse or mistreatment;
 6. Condoning abuse or mistreatment;
 7. Financial exploitation of a patient;
 8. Involuntary seclusion of a patient for the convenience of the staff or to discipline the patient;
 9. Wrongful use of a physical restraint upon a patient, excluding an act of restraint prescribed by a physician licensed under ORS chapter 677, physician assistant licensed under ORS 677.505 to 677.525, or nurse practitioner licensed under ORS 678.373 to 678.390, and any treatment activities that are consistent with an approved treatment plan;
(NOTE: Restraint used in accordance with OSH Policy 6.003, "Seclusion and Restraint" is not considered to be abuse or mistreatment.)
 10. Chemical restraint;
 11. Any act that constitutes a crime per statute, including ORS 163.375, 163.405, 163.411, 163.415, 163.425, 163.427, 163.465 or 163.467; and
 12. Any death of a patient caused by other than accidental or natural means or occurring in unusual circumstances.
- C. Abuse and mistreatment allegations are investigated by the Office of Training, Investigations, and Safety (OTIS). All types of prohibited conduct will be examined as part of the OTIS investigation.
- D. Staff must report patient allegations of abuse or mistreatment as delineated in this policy and other applicable regulations. Any staff who witnesses or has reasonable cause to believe that a patient has been abused or mistreated at OSH must immediately report the incident directly to OTIS at 503-945-9495 or otis-osh.reporting@odhsoha.oregon.gov.
- E. Staff must also notify the Superintendent or the Administrator on Duty via hand-delivered memorandum, email, telephone call, or via the Superintendent's Assistant during regular work hours. Staff may report after-hours to the Superintendent via the OSH Reception, who will forward the report to the Superintendent. Any person receiving such a notification must maintain its confidentiality as required by this policy and Oregon law.
- F. If staff has reasonable cause to believe that an OSH patient was the victim of abuse when the patient was a child or a resident of a nursing home or other healthcare facility, they must report that information to the child abuse and adult abuse hotline SAFELINE at 855-503-SAFE (7233), unless an exception below applies.
- G. If a psychiatrist, psychologist, or clergy receives information about child abuse or adult abuse during communications that are subject to the doctor-patient, psychotherapist-patient, or penitent privilege, the psychiatrist, psychologist, or clergy does not have a duty to report the alleged abuse or mistreatment unless

the alleged abuse or mistreatment occurred while the patient resides at OSH, regardless of the privilege.

- H. If staff reasonably believes that information about alleged child abuse is already known by a law enforcement agency or the Department of Human Services, they are not required to report the alleged abuse (see ORS 419B.010 [2]).
- I. Any person who in good faith reports alleged abuse or mistreatment and who has reasonable grounds for reporting has immunity from any criminal or civil liability pursuant to ORS 430.753(1), ORS 419B.025, and ORS 124.075.
- J. Staff may not retaliate against any person who reports suspected abuse or mistreatment in good faith. Any staff or other person who retaliates against any person because of a report of suspected abuse or mistreatment may be liable and subject to discipline.
- K. Persons or entities receiving confidential information pursuant to this policy must maintain the confidentiality of the information as required by state or federal law. The identity of the person reporting alleged abuse is confidential and may only be released as permitted by ORS 430.763, ORS 430.753(2), ORS 419B.035, or ORS 124.090.
- L. After a report of alleged abuse has been made, the following steps must be completed to enhance the investigation and protect patients:
 - 1. The Superintendent or their designee will implement protective measures as appropriate.
 - 2. If the allegation is determined to not fit the definition of abuse or mistreatment, the Superintendent or designee may take other appropriate action such as referral to Human Resources for review as a potential performance issue.
 - 3. Copies of all OTIS investigation reports must be maintained by the Superintendent in a place separate from employee personnel files.
- M. Upon completion of the OTIS investigation report, the Superintendent or designee must immediately notify the following, stating whether the allegation was substantiated, unsubstantiated, or closed without abuse determination:
 - 1. the patient or guardian;
 - 2. the staff or other person accused of abuse or mistreatment; and
 - 3. the Oregon Health Authority (OHA) designee.
- N. Staff may be subject to disciplinary or other appropriate action up to and including dismissal if found responsible for:
 - 1. abusing or mistreating a patient;
 - 2. failing to report abuse or mistreatment when they have reasonable cause to believe that abuse or mistreatment occurred;
 - 3. refusing to give information or giving untruthful information during an

- investigation of alleged abuse or mistreatment; or
4. retaliating against someone who makes a good faith report as required by this policy.
- O. In all situations where abuse, mistreatment, or failure to report abuse or mistreatment has been substantiated, the consequences must be commensurate with the seriousness of the conduct and any aggravating or mitigating circumstances and may include consideration of previous conduct of record (e.g., official personnel file).
1. Any staff found violating this policy may be referred to the appropriate law enforcement or regulatory agency.
 2. Any contractor found violating this policy is at risk of immediate termination of the contract.
 3. Any employee of the contractor found in violation of this policy may be excluded from OSH grounds and may be referred to the appropriate law enforcement or regulatory agency.
- P. If the accused is a licensed independent practitioner, the Superintendent's Office must report a substantiated allegation of abuse or mistreatment to the Medical and Allied Health Professional Staff Executive Committee.
- Q. Any staff dismissed or separated for violating this policy may not be rehired by OSH in any capacity. The staff person or former staff may not be allowed to visit or otherwise have contact with a current patient.
- R. Upon admission, each patient must be informed of this policy.
- S. Each staff must be provided a copy of this policy during new employee orientation. Each staff must sign a form acknowledging initial receipt of this policy.
- T. OSH follows all applicable regulations, including federal and state statutes and rules; Oregon Department of Administrative Services (DAS), Shared Services, and Oregon Health Authority (OHA) policies; and relevant accreditation standards. Such regulations supersede the provisions of this policy unless this policy is more protective of patients.
- U. Staff who fail to comply with this policy or related policy attachments or protocols may be subject to disciplinary action, up to and including dismissal.

III. DEFINITIONS

- A. "Abuse or mistreatment" means any act or absence of action toward a patient by staff inconsistent with prescribed treatment and care that falls within the definitions of abuse found in Oregon Revised Statute (ORS) 419B.005, ORS 430.735, ORS 124.050, and OTIS OAR 419-120-0010. Specifically, "abuse and

mistreatment” mean one or more of the following:

1. Abandonment, including desertion or willful forsaking of or the withdrawal or neglect of duties and obligations owed by a caregiver.
2. Any physical injury caused by other than accidental means, or that appears to be at variance with the explanation given of the injury.
3. Willful infliction of physical pain or injury.
4. Sexual abuse as defined by ORS 430.735(13).
5. Neglect.
6. Verbal abuse.
7. Financial exploitation.
8. Involuntary seclusion for the convenience of the caregiver or to discipline the adult.
9. A wrongful use of a physical or chemical restraint, excluding an act of restraint prescribed by a physician and any treatment activities that are consistent with the patient’s treatment plan or in connection with a court order.
10. An act that constitutes a crime under ORS 163.375, 163.405, 163.411, 163.415, 163.425, 163.427, 163.465 or 163.467.
11. Any death caused by other than accidental or natural means.
12. The restraint or seclusion of an adult with a developmental disability in violation of ORS 339.288, 339.291 or 339.308.
13. The infliction of corporal punishment on an adult with a developmental disability in violation of ORS 339.250 (9).

- B. “Closed without abuse determination” means after diligent efforts have been made, the investigator is unable to locate the accused person, the alleged victim, or another individual who might have information critical to the outcome of the investigation; or relevant records or documents are unavailable so that the investigation cannot be completed.”
- C. “Inappropriate sexual behaviors” include Sexual contact, Intimate/inappropriate touching, Kissing, Extended hand holding, Full body hugs, Hugs from behind, Sexual conversations/statements, Going off alone with another patient to be more intimate, Other behaviors normally associated with sexual interactions or relationships, and/or Any sexual or dating behavior the interdisciplinary treatment team deems as contraindicated for recovery
- D. “Licensed independent practitioner” in this policy means any professional staff who is permitted by law and by the hospital to provide patient care services without direction or supervision within the scope of his or her license and in accordance with individually granted clinical privileges.
- E. “Not substantiated” means a preponderance of the evidence does not support a conclusion that abuse, or mistreatment occurred.
- F. “Reasonable cause” means reasonable suspicion.

- G. "Self-defense" as defined in ORS 430.768 and used in OAR 407-045-0475(3) means the use of physical force upon another person in self-defense or to defend a third person. When excessively severe methods of control are used, or when any conduct designated as self-defense is carried beyond what is necessary under the circumstances to protect the patient or other persons from further violence or assault, that conduct then becomes abuse.
- H. "Sexual assault" means an incident of sexual contact between patients, patient to staff, staff to patient or non-consensual staff to staff where criminal activity is alleged to have occurred as defined by Oregon Criminal Code, including, but not limited to, non-consensual sexual intercourse or penetration, and those acts involving an alleged victim who lacks capacity to consent to sexual contact. (If any sexual behavior occurs in which one party is a staff and the other is a patient, it is always considered assault or abuse, even if the patient asserts that they consented.)
- I. "Sexual contact" is defined by ORS 163.305 and means any touching of the sexual or other intimate parts of a person or causing such person to touch the sexual or other intimate parts of the actor for the purpose of arousing or gratifying the sexual desire of either party.
- J. "Staff" includes employees, volunteers, trainees, interns, contractors, vendors, and other state employees assigned to work at Oregon State Hospital (OSH).
- K. "Substantiated" means a preponderance of the evidence supports a conclusion that abuse or mistreatment occurred.

IV. RELATED OSH POLICIES AND PROTOCOLS

OSH Policy 8.019 Staff Response to Alleged Criminal Acts
OSH Policy 1.003 Incident Reporting
OSH Policy 7.005 Patient Rights
OSH Policy 6.003 Seclusion and Restraints
OSH Policy 6.016 Sexual Activity Involving Patients
OSH Policy 5.009 Staff and Patient Relationships
OSH Policy 8.001 Video Surveillance

V. REFERENCES

42 CFR § 482.13(c)3.
42 CFR § 482.13(e).
45 CFR § 164.412.
Joint Commission Resources, Inc. (2021). *The joint commission comprehensive accreditation manual for behavioral health care and human services*, CTS.02.02.05. Oakbrook Terrace, IL: Author.
Joint Commission Resources, Inc. (2021). *The joint commission comprehensive accreditation manual for hospitals*, RI.01.06.03. Oakbrook Terrace, IL: Author.
Joint Commission Resources, Inc. (2021). *The joint commission comprehensive accreditation manual for hospitals*, PC.01.02.09. Oakbrook Terrace, IL: Author.
Oregon Administrative Rule § 407-045-0000.
Oregon Administrative Rule § 943-0450-0260.
Oregon Health Authority. *Individual privacy: Permissible and prohibited use and disclosure of information*,

OHA-100-003. Author.

Oregon Revised Statute §§ 124.050 — 124.095.

Oregon Revised Statute § 179.505.

Oregon Revised Statute §§ 419B.005—419B.050.

Oregon Revised Statute §§ 430.731 — 430.768.

Oregon Revised Statute § 441.044.

Oregon Revised Statute §§ 441.630 — 441.671.